



Profit Margins and Growth Potential

Three scenarios for the proposed Reserve Clinic peptide and supplement business, with the targeted media plan behind them

Prepared for review by the proposed brand partner & the project owner
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Proposed concept. The brand partner's and the medical co-founder's participation is unverified and subject to definitive agreements. Nothing in this document is an offer, an endorsement, or a representation that anyone has agreed to anything. Every figure is a model output produced by stated assumptions, not a forecast. Not legal, medical, tax or investment advice.

1. The answer, in one page

This version corrects a real error in the previous one. The earlier draft carried a venture-scale overhead on top of a plug-and-play operation: \$45,000 a month of team from launch month, \$100,000 of legal across four months, \$12,000 a month of creative and a \$150,000 launch push. That is \$104,000 a month of fixed cost against a business that was already producing \$144,000 a month of revenue at a 60% gross margin in month three. Break-even was being delayed by the assumed cost base, not by the unit economics or the demand. Corrected for the structure actually described — a \$600 provider carrying clinical operations, fulfilment and compliance administration, a founder plus one capable media buyer, and a partner who supplies much of the creative as a by-product of posting — **the business is EBITDA-positive in month three and needs about \$265,000 of capital.**

	Conservative	Baseline	Aggressive	Baseline, no partner
Net revenue, year 1	\$370k	\$4.2M	\$33.3M	\$580k
Net revenue, year 3	\$710k	\$14.9M	\$311.5M	\$3.9M
Net revenue, 36 months	\$1.6M	\$28.7M	\$499.9M	\$6.2M
Blended gross margin, year 3	50%	58%	66%	57%
EBITDA, year 3	(\$377k)	\$2.7M	\$118.1M	\$321k
EBITDA margin, year 3	-53%	18%	38%	8%
First profitable month	Not within 36 months	month 3	month 3	month 23
Capital required	\$1.2M	\$265k	\$408k	\$1.1M

Source / label: outputs/09_ReserveClinic_Margin_and_Growth_Model.xlsx, Summary sheet. Model outputs, not forecasts. The no-partner column holds every driver identical to Baseline except the ones the partnership supplies.

The margin answer

Margins are genuinely good, for two different reasons. The supplement line keeps about 60% of each order after product cost, shipping and card fees — \$46 of contribution on a \$78 order. The prescription line keeps less of each dollar, about 47%, but each dollar is larger and it recurs: \$116 per member per month. A supplement subscriber is worth roughly \$653 in gross lifetime contribution, a prescription member roughly \$1,055. Both clear their acquisition cost comfortably.

The growth answer

Baseline: \$4.2M of year-one revenue with \$571 thousand of year-one EBITDA, reaching \$14.9M at a 18% EBITDA margin by year three, on about \$265 thousand of capital, with 12,989 supplement subscribers and 1,219 prescription members at month 36. Cumulative cash turns positive in month 9, which means the business funds its own growth from there and the capital figure is a bridge rather than a burn.

Aggressive is the case the comparables say is possible: \$311M of year-3 revenue at a 38% EBITDA margin. For scale, IM8 — co-founded by David Beckham, whose Instagram audience is roughly three times larger — reportedly reached \$100 million in eleven months. This case reaches \$33M in year one, which is below that benchmark scaled for audience size. Treat the year-three EBITDA margin as the **maximum available** rather than a prediction: any operator seeing those numbers would reinvest the surplus into growth, and the model simply banks it.

Conservative is a stall, not a catastrophe. The business reaches about \$67 thousand a month and loses roughly \$30 thousand a month doing it — it never quite clears the compliance-grade overhead that a licensed telehealth-adjacent brand has to carry. Read section 7 before reading too much into it: that case requires every driver to land at its low end simultaneously, and section 7 shows what happens when they fail one at a time instead.

Why it is quarter one, not month ten

The unit economics were never the constraint. A supplement order carries \$46 of contribution, so the whole fixed cost base is covered by a few hundred orders a month. What changed is the denominator:

Monthly fixed cost	Previous draft	Corrected	Why
Team	\$45,000	\$16,000	A founder or GM, one capable performance-media buyer, part-time support. The provider carries clinical operations, fulfilment and compliance administration. Anything more is rebuilding in-house what the \$600 fee already covers.
Legal and regulatory	\$25,000 x 4 months	\$15,000 x 3 months	Still front-loaded, because it gates launch: entity formation, provider and pharmacy agreement review, claims and label review, endorsement protocol, paid-media likeness rights, trademark. But a one-off project cost, not a standing function.
Creative and content	\$12,000	\$6,000	A founder-partner posting several times a month supplies usable creative as a by-product. That saving is one of the concrete economic benefits of the partnership, and the previous draft did not credit it.
Launch push (one-time)	\$150,000	\$60,000	The point of a founder with a large audience is that the launch does not have to be bought. Spending heavily here pays twice for the same attention.
Total fixed base, month 3	\$103,880	\$56,580	Including the partner retainer, provider fee, certification, technology and insurance. Excluding paid media.

What did **not** change: the provider fee, certification, insurance, the medical director for the prescription line, and the percent-of-revenue floors that stop team, creative and technology staying flat as the business scales. Those are the items that make the business insurable, bankable and sellable, and cutting them would be cutting the reason the structure is defensible rather than cutting fat.

The one startup cost you do have

'Almost zero startup costs' is right about overhead and wrong about working capital. The cash trough in the baseline case is \$204 thousand, and it is almost entirely inventory rather than operating loss. Supplement manufacturing is paid up front, in batch runs, against minimum order quantities, weeks before the revenue arrives:

Model month	EBITDA	Cash into inventory	Net cash	Cumulative cash	Inventory on hand
Month 1	(\$37k)	\$0	(\$37k)	(\$37k)	\$0
Month 2	(\$84k)	(\$61k)	(\$145k)	(\$182k)	\$61k
Month 3	\$26k	(\$48k)	(\$22k)	(\$204k)	\$109k
Month 4	\$46k	(\$28k)	\$18k	(\$186k)	\$137k
Month 5	\$48k	(\$24k)	\$25k	(\$161k)	\$161k

Model month	EBITDA	Cash into inventory	Net cash	Cumulative cash	Inventory on hand
Month 6	\$57k	(\$22k)	\$35k	(\$126k)	\$182k

Operating losses total only \$121 thousand across the first two months and turn positive in month three. The inventory build is what keeps cumulative cash negative until month 9, and it keeps growing with revenue: a faster-growing business ties up more cash, not less. The prescription line has none of this problem — the pharmacy holds that inventory — which is another argument for launching the supplement line first with a tight initial run and reordering into demand rather than ahead of it.

The partnership answer

Comparing Baseline against the identical business with no partner: **+\$22.6M of 36-month revenue, +\$5.4M of 36-month EBITDA, break-even 20 months earlier, and \$0.9M less capital to get there.** Against that, the illustrative partner package costs \$1.95M in cash over 36 months. The partnership pays for itself roughly two to one on EBITDA alone, before any equity. Section 6 shows the workings.

The most useful thing here is still a sequence, not a number. The supplement line can trade in all fifty states, needs no prescriber, carries the better margin, and is the only line the brand partner can promote. The prescription line needs the provider, the pharmacies, the state-by-state licensure map and a medical director, and it cannot be promoted by a celebrity. Launch the first and earn the right to the second.

2. This is two businesses, and they behave differently

Treating the venture as one company with one margin is the most common way these models go wrong. The two lines have different legal footprints, cost structures, growth ceilings and promotional rules. The model keeps them separate throughout.

	Non-prescription supplement line	Prescription telehealth line
What it is	DSHEA-compliant supplements and topicals sold through an open consumer checkout.	Compounded peptide programmes prescribed after a clinical consult and dispensed by a licensed pharmacy.
Geographic reach	All fifty states from day one.	Starts at 36% of the US population and ramps toward a 82% ceiling. Fifty-state prescription coverage is not available on day one.
Gross margin (year 3, baseline)	60%	47%
Unit of value	\$46 contribution per order; \$653 per subscriber over their life.	\$116 contribution per member-month; \$1,055 per member over their life.
Main risk	Retention, and the cost of paid acquisition once the audience effect fades.	Retention, which is materially worse in this category, plus regulatory dependence on which peptides remain compoundable.

	Non-prescription supplement line	Prescription telehealth line
Can the brand partner promote it?	Yes, with clear and conspicuous disclosure of the material connection on every post.	No. Specific compounded prescription products should not be promoted by a celebrity partner. This is a design constraint, not a preference.
Can partner creative run in paid ads?	Yes, subject to paid-usage likeness rights and platform health-category ad review.	No. Prescription intent is therefore weighted down on partner-driven traffic and comes from search, editorial content and on-site cross-sell.
Share of year-3 revenue (baseline)	79%	21%

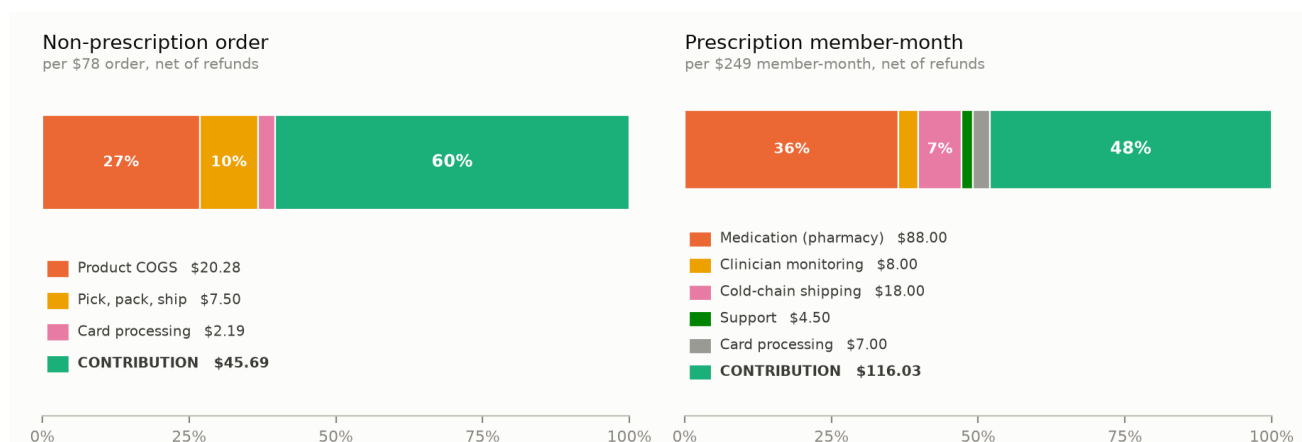
Source / label: Model outputs plus the regulatory constraints in research/07_margin_growth_benchmarks.json (B18). Every legal statement here needs confirmation by qualified FDA, pharmacy, corporate-practice-of-medicine and advertising counsel before it is relied on.

Why the prescription line cannot simply be switched on nationally

- **Licensure and asynchronous prescribing.** A clinician must be licensed in the patient's state, and states differ on whether a questionnaire-based visit establishes a prescriber-patient relationship. Some states are excluded from an asynchronous model outright.
- **Corporate practice of medicine.** Many states bar a non-physician company from owning a medical practice or sharing professional fees. The prescribing entity has to be a separate physician-owned company paid a flat, fair-market management fee — never a share of revenue or a payment per prescription.
- **The five percent interstate cap.** Section 503A(b)(3) limits a compounding pharmacy to distributing 5% of its prescription orders across state lines unless its state has signed FDA's memorandum of understanding. FDA's compounding MOU index currently lists three MOUs, all suspended, and FDA has repeatedly deferred enforcement of the 5% limit pending rulemaking. Deferred enforcement is discretionary and revocable, so a national single-pharmacy prescription model is a policy bet rather than a settled structure. A multi-state pharmacy network is the durable answer.

3. Where the margin comes from

Both charts show where each dollar of net revenue goes at baseline drivers. Nothing is allocated or averaged; these are the per-unit costs incurred on every order and every fill.



Source / label: Baseline column of the Assumptions sheet. Non-prescription costs benchmarked to a 20-30% landed-COGS band (B03); prescription costs to compounded-peptide programme pricing (B07), asynchronous consult cost (B08) and cold-chain fulfilment quotes (B09).

The three costs that decide the prescription margin

Cost	Baseline	Negotiable?	What moves it
Pharmacy medication, per member-month	\$88	Yes, at volume	A written tiered schedule. The largest single variable cost in the model, and the first thing to get in writing.
Cold-chain fulfilment and shipping, per fill	\$18	Barely	Temperature-controlled two-day service has a floor. Longer refill cadences — 90-day supply instead of 30 — cut the number of cold shipments and are the real lever.
Clinician monitoring, per member-month	\$8	Somewhat	Must be paid per unit of clinical work, never as a share of revenue. That constraint is not negotiable.

The shipping cadence is the most under-appreciated margin lever in the model. Moving a member from a 30-day to a 90-day fill removes two cold-chain shipments per quarter — \$36 of cost per member per quarter, roughly 5% of quarterly revenue recovered with no price change and no new customer. Whether the formulary and stability data allow it is a clinical and pharmacy question worth asking early.

What the flat \$600 monthly provider fee does and does not change

The flat fee is genuinely powerful: it removes almost all fixed operating cost. A conventional version of this business carries a telehealth platform, an EHR, a provider network, pharmacy operations and a compliance function as monthly overhead. Here that is \$600.

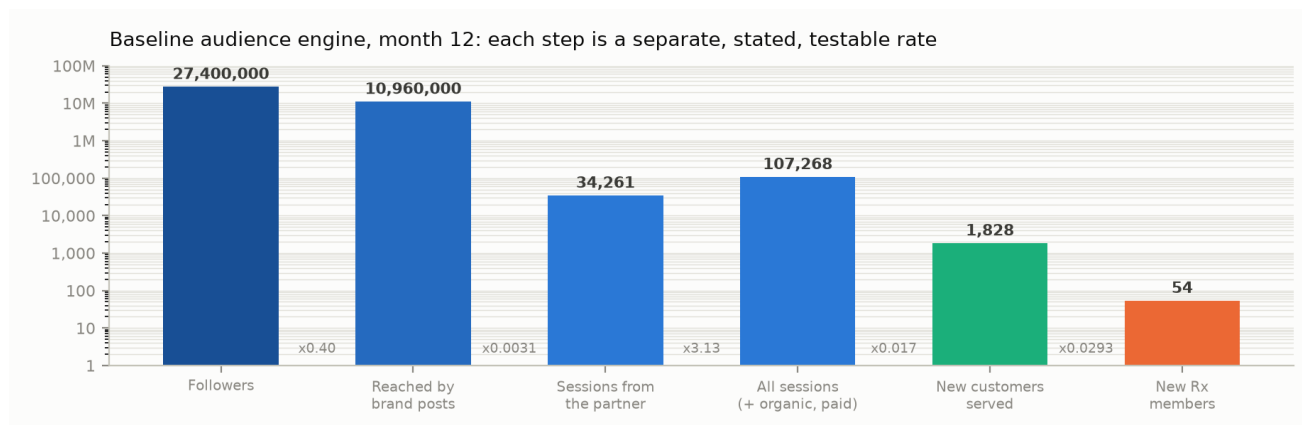
It does not remove variable cost, and the model does not assume it does. Medication, per-consult clinician time, cold-chain shipping and card processing all scale with every order and every fill, and each is carried as a separate driver. The reason for that caution is specific: published pricing for comparable turnkey telehealth infrastructure runs \$3,000 to \$6,000 per month by tier, plus \$5,000 to \$10,000 of onboarding, plus per-consult fees. A \$600 flat fee sits at roughly one fifth of the low end of that range.

That gap has two explanations and both lead to the same action. Either the fee covers software and compliance administration only — how this model treats it, and the safe reading — or the scope is narrower than expected. Get the fee schedule in writing. The three questions that matter most: are per-consult fees included, does the pharmacy bill per fill, and who holds the LegitScript certification and for which website. The workbook's Provider_Diligence sheet has the other seventeen.

One further point. At \$600 a month the provider carries no volume risk and has no economic stake in growth. Excellent for margin, poor for alignment: nothing in the fee structure funds the provider's own scaling, so consult turnaround, fill turnaround, capacity ceilings and what happens when they are exceeded all need to be contractual rather than assumed.

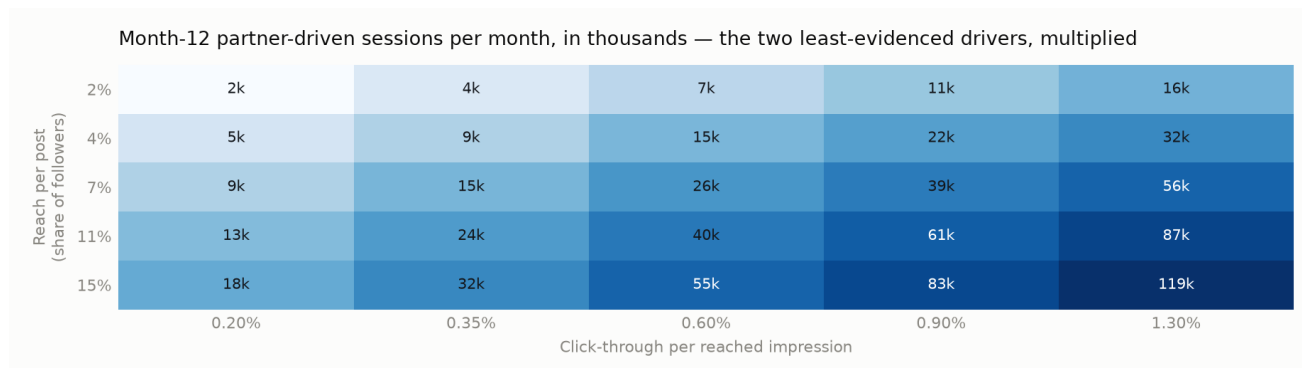
4. The growth engine is the audience, and it has four dials

Most of the spread between scenarios comes from four numbers, multiplied together. Stating them separately is the point: each is measurable within weeks of going live, so the band narrows fast once there is real data.



Source / label: Baseline drivers, model month 12. Follower count from third-party trackers (B16): 27,410,219 as of June 2026, range 27.0M to 28.3M across sources. Not an audited or first-party figure.

Dial	Cons.	Base	Aggr.	Evidence
Brand posts per month	3	5	7	A contract deliverable, not a market variable.
Reach per post, share of followers	5.5%	8.0%	10%	Analyst assumption. The 10M-plus tier averages about 1.77% ENGAGEMENT (B15); reach exceeds engagement, but organic reach at this tier is heavily throttled and branded content more so.
Click-through per reached impression	0.50%	0.70%	0.95%	Analyst assumption. No tier-level link-click benchmark was retrievable. The least evidenced number in the model.
Order conversion, all sessions	1.0%	1.5%	2.0%	DTC supplement conversion commonly 0.5-1.5%. A warm follower audience should sit at the top of that band, though sources warn mega-tier audiences convert worse than their reach implies (B15).



Source / label: Sensitivity sheet, grid 2. Month-12 partner-driven sessions per month at baseline posting cadence and decay.

The grid is the honest version of the growth question. The answer moves by two orders of magnitude on two numbers nobody can yet measure. Any single-point projection for this business is a guess

dressed as an estimate.

Attention decays, and the model makes it decay

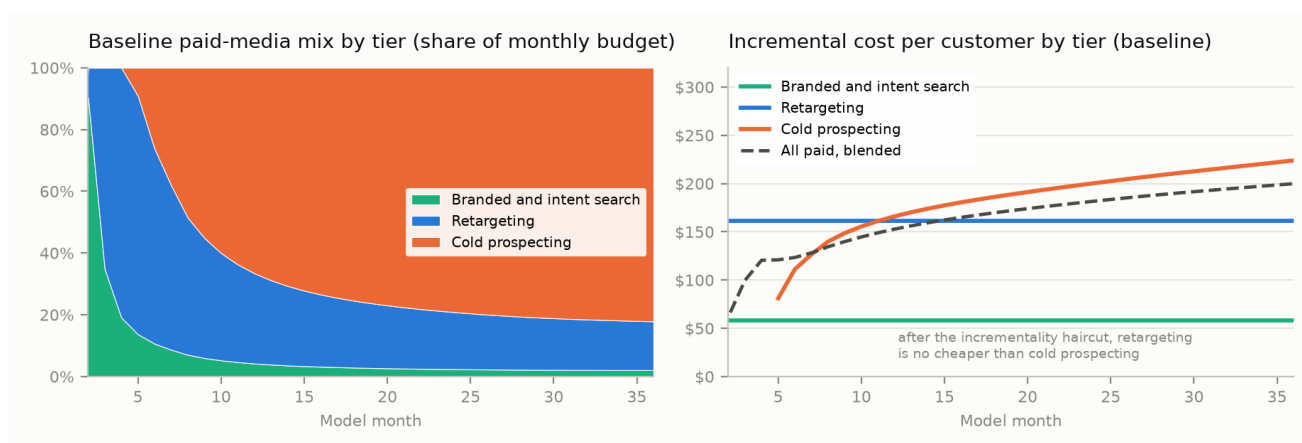
Partner-driven traffic is modelled with a launch spike of 2.3x, then monthly decay toward a floor of 38% of the launch peak — the durable follower demand that persists after novelty. A model without decay makes a celebrity launch look like a permanent acquisition channel, and it is not one. IM8's founders said publicly that they deliberately did not lean on the fame, but on whether the product worked. The floor is where product quality shows up in the arithmetic.

5. Targeted paid media: how the partnership actually lowers acquisition cost

A celebrity partner does not lower the price of advertising by magic, and a model that simply assumes a better CAC is not worth reading. It works through three specific mechanisms, each of which can be measured and each of which has a ceiling. The model treats paid media as three separate tiers rather than one blended number, sets the monthly budget as a share of the prior month's revenue, and fills it from the cheapest tier upward.

Tier	What it is	How the partnership creates it	What caps it
1. Branded and intent search	Paid search against the brand name and category-intent queries. The cheapest inventory a brand owns, because it faces almost no auction competition on its own name.	Every brand post sends some viewers to a search engine instead of to the link, converting reach into cheap high-intent paid inventory. Without the partner this tier is nearly empty.	The queries that exist. \$58 per customer at baseline — by far the best CAC in the plan, and by far the smallest volume.
2. Retargeting	Site visitors plus the partner's post engagers, reached again through platform custom and engagement audiences.	Anyone who engages with a brand post enters the platform's engagement audience, so partner reach builds a large warm pool at no media cost.	Pool size and fatigue. \$161 per incremental customer at baseline.
3. Cold prospecting with partner creative	Lookalike and broad audiences running creative that features the partner.	A recognisable face and partner-voiced video should raise click-through and so lower cost per session. The only tier where the partnership improves efficiency rather than creating volume.	Diminishing returns, not volume. \$200 per customer at baseline month 24, rising as spend grows.

Source / label: Paid_Media sheet. Tier costs and conversion multipliers from benchmark B19 (retargeting CPA 40-70% below cold; ROAS about 4.2x against 1.5-3x) and B20 (paid-social click-through and creative findings).



Source / label: Baseline scenario. Left: the cheap warm tiers dominate early, when the budget is small relative to the audience, then cold prospecting takes over as budget grows past what the warm pools can absorb. Right: incremental cost per customer by tier.

The incrementality haircut, and why it matters more than the headline discount

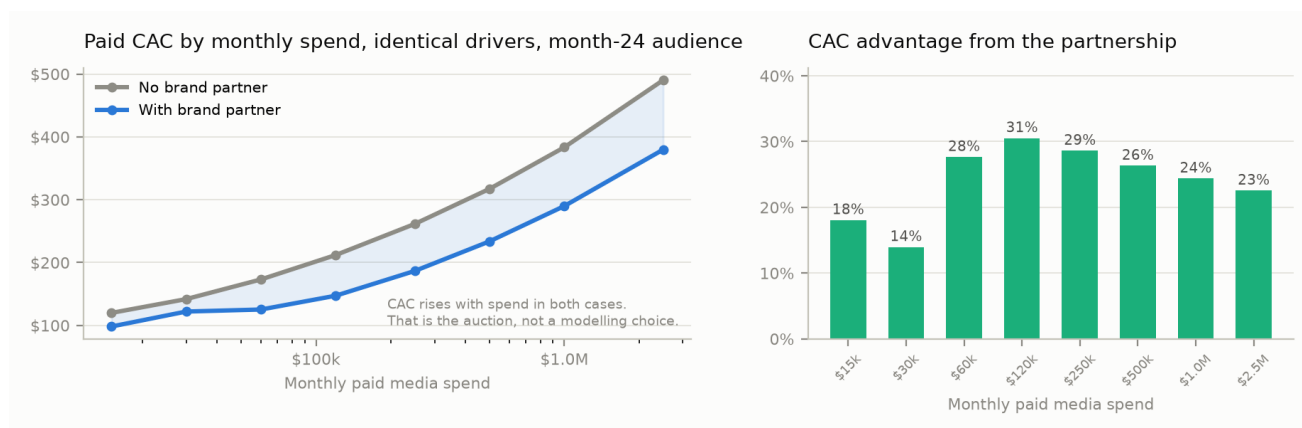
The widely quoted figures say retargeting cuts cost per acquisition by 40 to 70 percent against cold traffic. Taken at face value, that would make retargeting the obvious place to put the budget. The same sources contain the catch: holdout tests show true incremental lift of only 25 to 30 percent, meaning up to three quarters of retargeting conversions would have happened anyway. Retargeting looks efficient largely because it harvests demand that prospecting and the partner's organic posts already created.

The model therefore applies an incrementality haircut to both warm tiers: 30% on retargeting and 45% on paid branded search. The branded-search haircut is not optional either — paying for a query you would rank for organically buys a click you already had, and without the haircut the model would count the partner's organic traffic twice.

The consequence is counterintuitive and worth acting on. After the haircut, retargeting's incremental cost per customer at baseline drivers comes out at \$161 — essentially level with cold prospecting at \$200, despite a much cheaper nominal CPA. In the aggressive case, where the warm pool is far larger relative to spend, retargeting is genuinely cheaper (\$65 against \$172). So: **run a retargeting holdout test in month two.** If incrementality lands at the low end of the band, that budget belongs in prospecting instead. This is a fortnight of work that decides how a fifth of the media budget is spent.

So does the partnership buy a better CAC? Yes — at a given level of spend

This is the like-for-like test. Both lines below use identical drivers, prices and conversion rates. They differ only in the audience conditions the partnership creates: partner impressions, and the warm pools those impressions build.



Source / label: Sensitivity sheet, grid 4, at month-24 audience conditions. Computed closed-form from the Baseline and no-partner assumption columns, so it recalculates with the workbook.

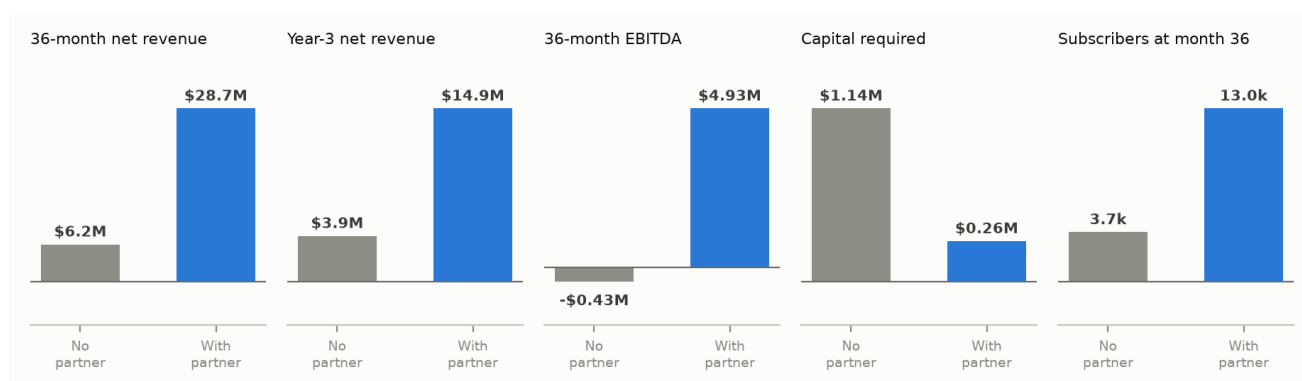
Monthly paid spend	CAC with partner	CAC no partner	Advantage	Advantage %
\$15k	\$98	\$120	\$22	18%
\$30k	\$122	\$142	\$20	14%
\$60k	\$125	\$173	\$48	28%
\$120k	\$147	\$212	\$65	31%
\$250k	\$187	\$261	\$75	29%
\$500k	\$234	\$317	\$84	26%
\$1.0M	\$290	\$383	\$94	24%
\$2.5M	\$380	\$490	\$111	23%

The advantage runs 14% to 31% depending on spend level. Two features of that curve matter more than the headline:

- **CAC rises with spend in both cases.** That is the auction, not a modelling choice. Any model showing acquisition cost falling as a business scales has no competition in it. This is why the blended CAC in the summary table is higher for Baseline than for the no-partner case: Baseline is a much larger business spending far more money, operating further out on the same curve. Comparing the two at equal spend, as this grid does, is the only fair comparison.
- **The advantage is largest on the first tranche of spend and narrows as budget grows.** That is the correct reading of a celebrity partnership: a strong subsidy on early spend, not a permanent discount on all of it. It is also an argument for spending into it early and hard rather than trickling budget over three years.

6. What the partnership is worth

The fourth column of the model is the same business with no brand partner. Organic search, pricing, the cost structure, the provider, the team plan and the whole operating base are held identical. The only differences are the four things the partnership actually supplies: the partner's own social traffic, the branded-search and engagement-retargeting inventory that traffic creates, the creative lift on cold prospecting, and the partner's compensation.



Source / label: Summary sheet, Baseline against Baseline-no-partner. Holding organic and SEO identical is deliberate, and it understates rather than flatters the partnership: a brand without a famous founder would also generate less press and weaker organic growth, so the true gap is probably wider than shown.

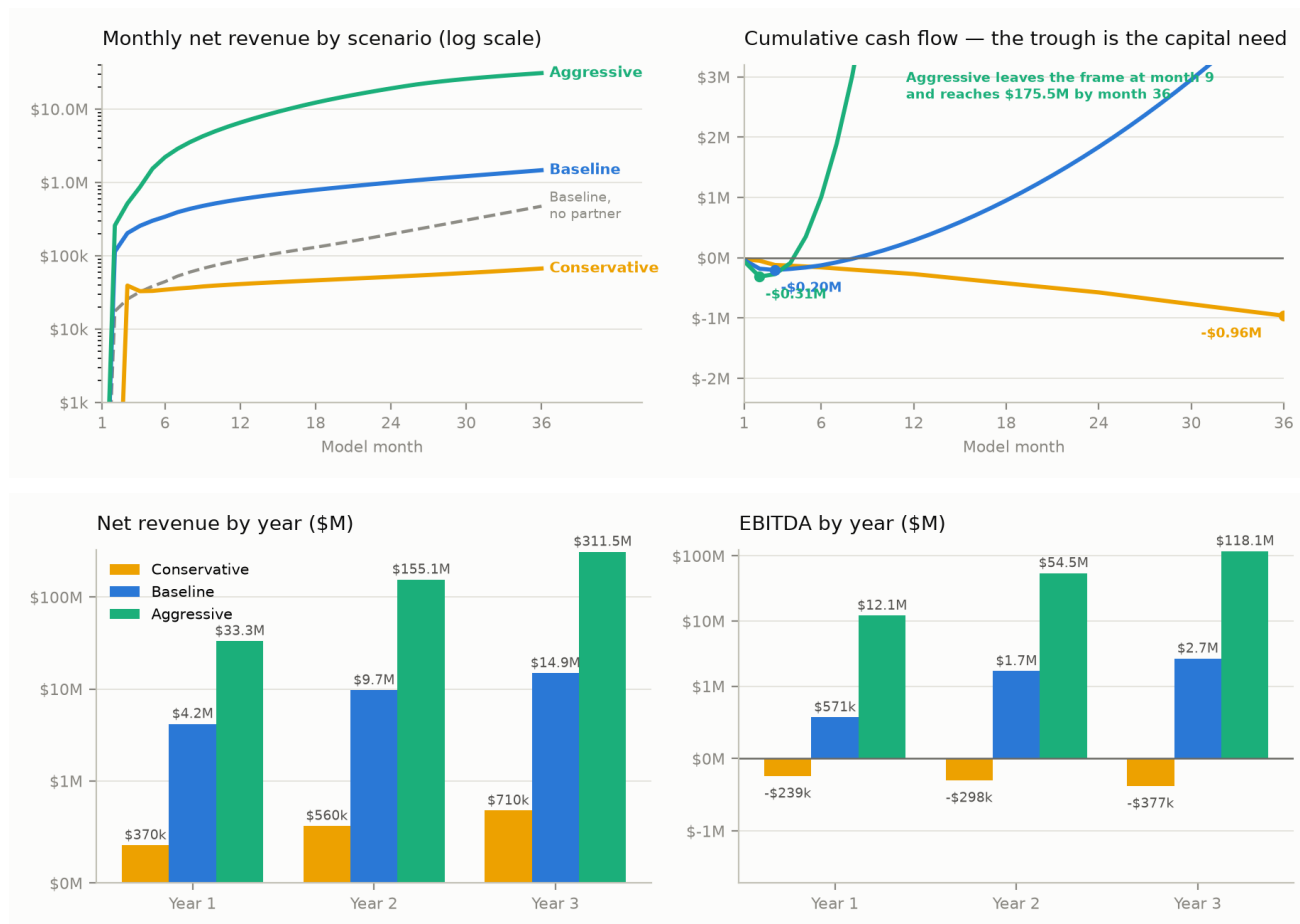
	Baseline, no partner	Baseline	Difference
Net revenue, year 1	\$580k	\$4.2M	\$3.6M
Net revenue, year 3	\$3.9M	\$14.9M	\$11.0M
Net revenue, 36 months	\$6.2M	\$28.7M	\$22.6M
EBITDA, 36 months	(\$435k)	\$4.9M	\$5.4M
First profitable month	month 23	month 3	20 months earlier
Capital required	\$1.1M	\$265k	(\$870k)
Subscribers at month 36	3,708	12,989	9,282
Prescription members at month 36	480	1,219	739

The arithmetic of the deal, at baseline. The partnership adds \$5.37M of 36-month EBITDA and removes \$0.87M of capital need. The illustrative package — a \$15,000 monthly retainer plus a 5% royalty — costs \$1.95M in cash over the same period. That is roughly a 2.8 to 1 return on the cash cost, before counting the equity, and it flips the business from cumulatively loss-making to cumulatively profitable. On these assumptions the partnership is not a marketing expense; it is the thing that makes the business work.

Two honest caveats on that conclusion. First, it rests entirely on the four audience dials in section 4, two of which are unmeasured — if reach and click-through land at the conservative end, the partnership adds far less. Second, the comparison assumes the partner's likeness can be licensed for paid media and cleared by platform ad review; if paid usage is restricted to organic posting only, the creative-lift mechanism and much of tier 3's advantage disappear while the cash cost does not.

7. The three scenarios

Same cost structure, same product, same provider, same media architecture. What differs is how much the audience delivers, how long customers stay, how fast the supply chain scales, and how disciplined the spend is.



Source / label: Model outputs. The left panel is log-scaled because the scenarios differ by roughly three orders of magnitude; the dashed grey line is Baseline with no partner. The cash panel is clipped at \$3M so the troughs stay legible.

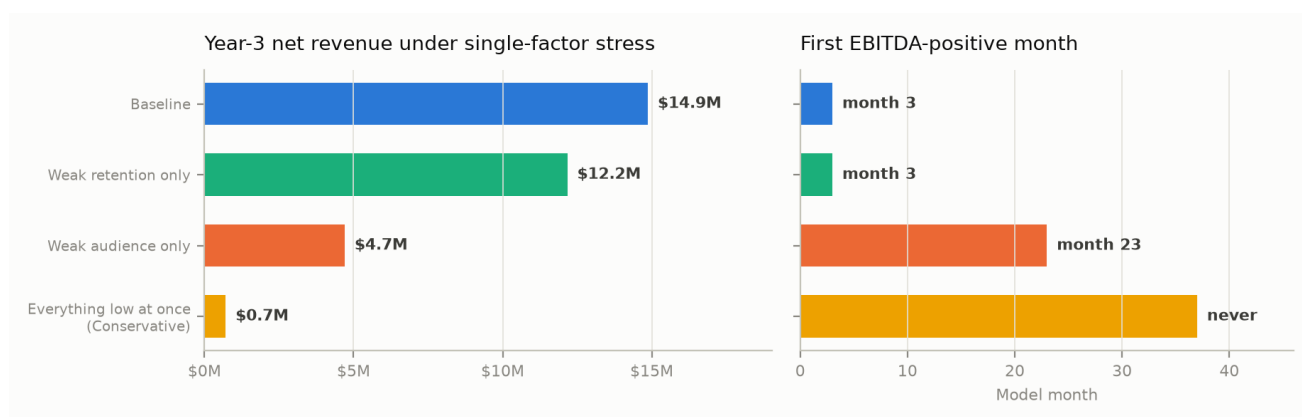
Metric	Conservative	Baseline	Aggressive	No partner
Net revenue, year 1	\$370k	\$4.2M	\$33.3M	\$580k
Net revenue, year 2	\$560k	\$9.7M	\$155.1M	\$1.7M
Net revenue, year 3	\$710k	\$14.9M	\$311.5M	\$3.9M
Gross profit, year 3	\$355k	\$8.6M	\$204.9M	\$2.2M
Blended gross margin, year 3	50.0%	57.7%	65.8%	56.9%
EBITDA, year 3	(\$377k)	\$2.7M	\$118.1M	\$321k
EBITDA margin, year 3	-53.0%	17.9%	37.9%	8.2%
EBITDA, 36-month cumulative	(\$914k)	\$4.9M	\$184.7M	(\$435k)
First EBITDA-positive month	Not within 36 months	month 3	month 3	month 23
Peak cumulative cash deficit	(\$961k)	(\$204k)	(\$314k)	(\$873k)

Metric	Conservative	Baseline	Aggressive	No partner
Capital required, incl. 30% buffer	\$1.2M	\$265k	\$408k	\$1.1M
Supplement subscribers at month 36	724	12,989	217,215	3,708
Prescription members at month 36	30	1,219	33,197	480
Blended CAC, year 3	\$86	\$112	\$121	\$93
Supplement LTV:CAC, year 3	4.4x	5.8x	8.7x	7.0x

Source / label: Summary sheet. Every cell traces to a formula over the Assumptions sheet; there are no hard-coded results. Note that blended CAC is HIGHER in the larger scenarios — see section 5 for why that is the expected result.

How wide should this band really be?

Conservative to aggressive spans 439 times on year-3 revenue, which is too wide to plan against, and it is worth being precise about why. Scenarios built by setting every driver to its low end do not describe a one-in-ten outcome; they describe something closer to one-in-a-hundred, because the pessimism compounds across eight independent assumptions. The useful test is to fail one factor group at a time and leave everything else at baseline.



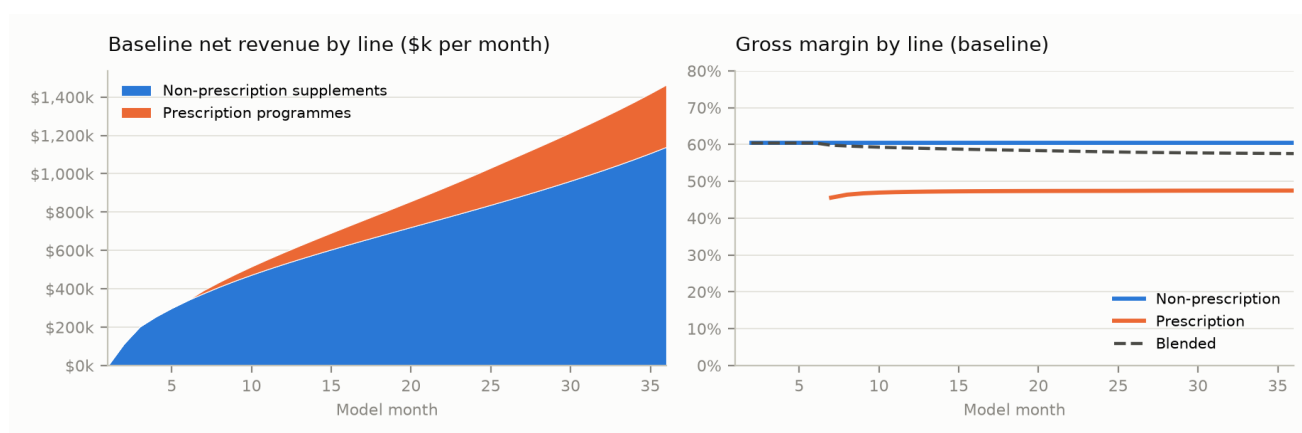
Source / label: Model_Weak_Audience and Model_Weak_Retention on the Summary sheet: identical to Baseline except that one factor group is moved to its Conservative values. Weak audience moves posting cadence, reach, click-through, attention floor, launch spike and site conversion. Weak retention moves both churn rates and the reorder rate.

	Year-3 revenue	Year-3 EBITDA	36-month EBITDA	First profitable month	Capital required
Baseline	\$14.9M	\$2.7M	\$4.9M	month 3	\$265k
Weak retention only	\$12.2M	\$2.1M	\$4.0M	month 3	\$265k
Weak audience only	\$4.7M	\$149k	(\$373k)	month 23	\$878k
Everything low at once (Conservative)	\$710k	(\$377k)	(\$914k)	Not within 36 months	\$1.2M

Two conclusions, and the second is the one to act on. First, no single factor sinks this. Retention landing at the conservative end costs 18% of year-three revenue and the business is still profitable from month 3 with \$4.0M of cumulative EBITDA. It takes the audience underperforming AND everything else going wrong to reach the conservative number. Second, **the audience is the whole ballgame.** Weak audience alone removes 68% of year-three revenue, pushes break-even from month 3 to month 23, and turns 36-month EBITDA negative. Everything else in this document is second-order by comparison — and the audience dials are measurable within sixty days.

So the practical planning range is baseline to aggressive, roughly \$15M to \$311M of year-3 revenue, with weak-audience as the downside to hold capital against rather than the compound-conservative case. The conservative column stays in the model because a lender or an investor will ask for it, not because it is the number to budget to.

	Conservative	Baseline	Aggressive
The story	Every driver lands at its low end at once. A stall: the brand reaches roughly \$67 thousand a month and does not clear its compliance overhead.	The audience performs at the middle of the plausible range. A real, profitable brand gets built on modest capital, with most of the value in the subscriber base.	The audience performs like the best celebrity supplement launches on record, retention holds, and the supply chain keeps up.
Monthly churn, supplements	9.0%	7.0%	5.5%
Monthly churn, prescription	13%	11%	9.5%
Prescription line goes live	month 9	month 7	month 5
Paid media, share of prior-month revenue	30%	24%	20%
Partner creative cost discount	0.88x	0.80x	0.72x
Retargeting incrementality	25%	30%	40%
First-month shippable order capacity	600	1,500	3,000
What would make you believe it	Month-one link-click data comes in at or below 0.35% of reached impressions.	Month-one click-through near 0.6%, month-three subscription churn at or under 7%, and a retargeting holdout showing incrementality at or above 30%.	Month-one click-through near 0.9%, conversion above 1.8%, and a manufacturer able to ship 3,000 orders in the first live month.

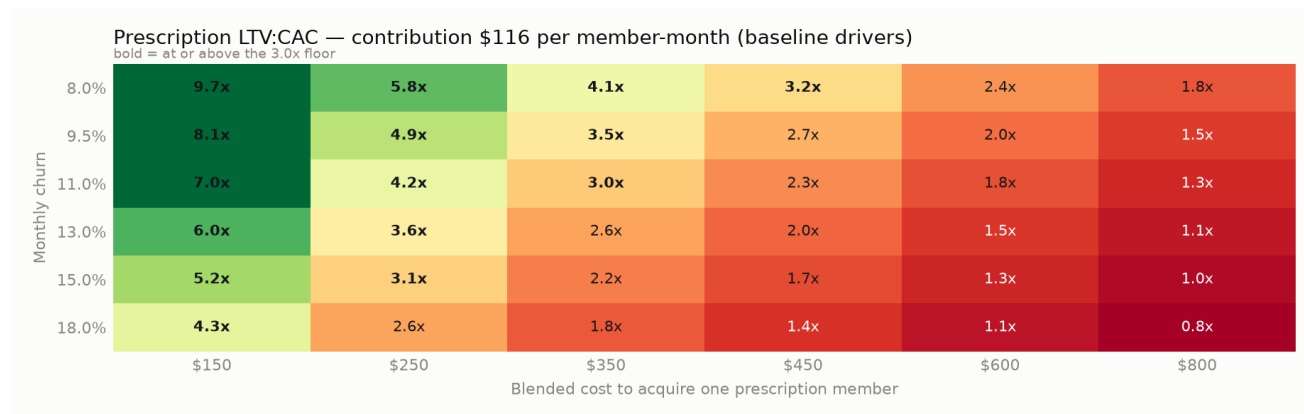


Source / label: Baseline scenario. The supplement line carries the revenue early and stays the majority of it; the prescription line adds revenue at a lower margin, which is why blended margin drifts down as it grows.

That downward drift looks like deterioration and is not. It is mix. Adding a 47%-margin prescription business to a 60%-margin supplement business lowers the average while increasing both total gross profit and revenue per customer. The question to ask of the prescription line is not whether it dilutes margin — it does — but whether its contribution per member covers its acquisition cost. At baseline it does, comfortably.

8. Retention is the thing most likely to break this

Of every assumption in the model, prescription churn is the one where the outside evidence is strongest and least encouraging. In the closest comparable population, discontinuation approaches 65% within twelve months and 84.4% by twenty-four. A real-world telehealth provider reported programme retention of 78%, 63% and 58% at three, six and nine months. Trade commentary describes a 2026 retention cliff as the category's central problem.



Source / label: Sensitivity sheet, grid 1, computed from the baseline drivers. LTV is contribution per member-month divided by monthly churn; the 3.0x floor is the conventional minimum for a healthy subscription business (B06).

At the baseline contribution of \$116 per member-month, the prescription line clears the 3.0x floor across a wide range of acquisition costs as long as churn stays near 11%. Above roughly 15% monthly churn it stops working at any plausible CAC. That is not a distant edge case: 15% monthly churn is what 84% annual discontinuation looks like.

Retention is not a metric to monitor, it is the product roadmap. Longer fill cadences, real clinical check-ins, outcome tracking and honest expectation-setting at intake are the levers. A brand that solves retention in this category has a durable business; one that does not has an expensive customer treadmill, whatever the gross margin says.

Four other things that would change the answer materially

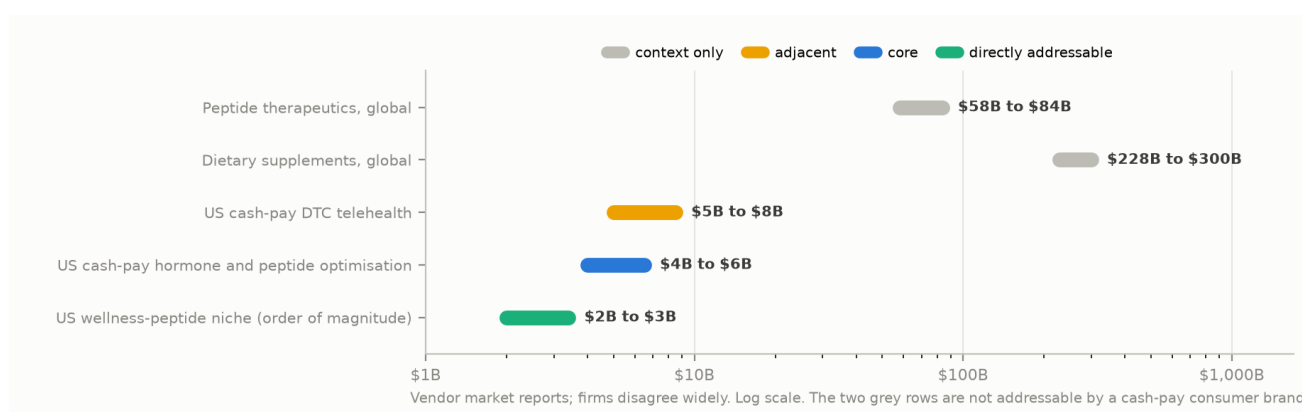
Risk	Why it bites	What to do about it now
Which peptides stay compoundable	The formulary is set by FDA's 503A bulk drug substances list, and it moves. FDA removed twelve peptides from Category 2 in April 2026; the advisory committee met in July 2026 on three more and will meet again before the end of February 2027 on five others. A product can become uncompoundable between signing and launch.	Verify the formulary against the live FDA list on the day you sign, not from any summary including this one. Put a tracking obligation on the provider in the contract. Do not build the revenue plan on a single molecule.
Paid-usage likeness rights	The whole tier-3 creative discount, and a good part of the partnership's measured value in section 6, assumes the partner's likeness can run in paid advertising. Organic posting rights and paid-usage rights are licensed separately. If paid usage is withheld, the cost of the deal stays and part of the benefit goes.	Put paid-media usage, territories, terms, approval workflow and post-termination tail in the agreement explicitly. Do not infer it from a content deliverable schedule.

Risk	Why it bites	What to do about it now
LegitScript certification	Certification is site-specific and gates healthcare advertising on Google, Meta, Microsoft and TikTok, and is close to a prerequisite for card-not-present acceptance. If the provider holds it on the provider's domain, your site may not be covered. Without it, most of the paid plan in section 5 cannot run at all.	Confirm in writing who holds it and for which website. Budget about \$3,100 in year one and \$2,150 a year thereafter per site, and confirm the live schedule.
Endorsement disclosure	Every paid or equity-holding endorsement requires a clear and conspicuous disclosure of the material connection, on every post and in every format. A founder's equity stake is a material connection. Earlier research in this project found the proposed partner named on the FTC's September 2017 list of influencers sent warning letters about undisclosed endorsements, so this is a specific and personal exposure here rather than a generic one.	Write the disclosure protocol before the first post, not after. Require pre-publication review of brand content. Keep the partner away from prescription-product claims entirely.

Source / label: research/07_margin_growth_benchmarks.json entries B10, B15, B18, B20, plus the compliance matrix and celebrity-founder research in the main business plan. All of this requires confirmation by qualified counsel.

9. Category context, honestly labelled

Market-size numbers in this category are routinely misused, so here is the version with the labels attached. The large figures are not addressable by a cash-pay consumer brand; the addressable slice is the bottom two rows.



Source / label: Vendor market reports with wide disagreement between firms; 2026 estimate to 2030 estimate. Peptide therapeutics estimates alone range from \$54.6B to \$164B depending on scope. The bottom two rows are order-of-magnitude analyst estimates, not measurements.

Two things in that picture are useful. First, the direction and pace: cited growth of 7-11% a year across both the supplement and peptide categories means the tide is moving the right way, which is not true of every consumer category right now. Second, the directly addressable niche is a few billion dollars, large enough that even the aggressive case (\$311M of year-3 revenue) is a low single-digit share of it. The model does not assume category dominance.

What the comparables actually show

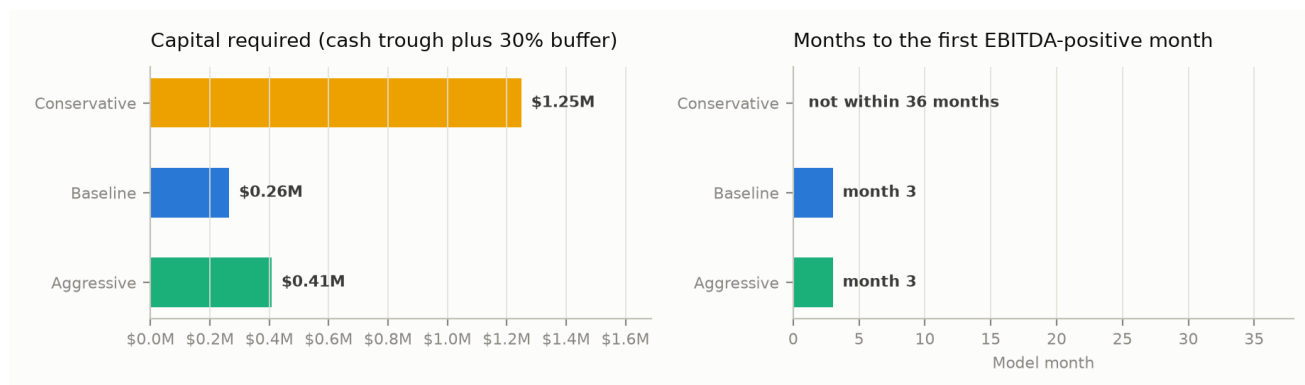
Comparable	Audience	Reported outcome	What it tells us
IM8 (David Beckham)	~88M Instagram followers	Reportedly \$100M in eleven months; about \$120M ARR in year one.	The aggressive case is achievable. Note the founders stated they deliberately did not rely on the fame but on whether the product worked.
Gruns	Celebrity-backed, creator-led	Profitable fourteen months in; nine-figure ARR; a reported \$500M valuation.	Profitability at scale is reachable, but it took over a year even with a strong launch.
SHREDZ	Influencer-led	\$90k in its first year (2012); \$5M the following year.	The downside case is real. The spread between SHREDZ and IM8 is roughly three orders of magnitude, which is exactly why the scenario band here is wide.
Hims & Hers	Listed telehealth, no celebrity founder	74% gross margin in FY2025 falling to 63.8% in Q2 2026; marketing about 35-40% of revenue.	The margin ceiling for telehealth at scale, and a warning: their margin fell as they added fulfilment-heavy weight-loss offerings. Ours carries cold chain from day one.

Source / label: Press reporting and self-reported revenue for the private companies; company filings and earnings releases for Hims & Hers. None of the private figures is audited. Full citations in research/07_margin_growth_benchmarks.json entries B01, B02 and B14.

The Hims & Hers row is the only audited comparable and it carries a specific warning. Their gross margin fell roughly ten points as weight-loss offerings with shorter shipping cadences and higher fulfilment costs grew into the mix. That is precisely the dynamic our prescription line has from day one, and the reason this model carries cold-chain shipping as an explicit per-fill cost rather than burying it in a blended margin.

Their marketing ratio is also the benchmark for section 5. Hims runs marketing at 35-40% of revenue with no celebrity founder. This model runs paid media at 24% of revenue at baseline and 20% at aggressive. **If that ratio has to climb toward 35% in practice, the partnership is not doing its job** — and that single ratio is the cleanest ongoing test of whether the audience is working.

10. What it costs to find out



Source / label: Capital required is the deepest cumulative cash trough in each scenario plus a 30% buffer. It excludes any partner cash beyond what is modelled, and excludes a valuation event.

Three things in that picture are worth naming. First, **the capital requirement is lowest in the cases that work** — not a modelling error. Partner-driven traffic carries no media cost, the cheap search and retargeting tiers absorb the early budget efficiently, and the business turns EBITDA-positive in month three, so revenue arrives before most of the spending does. Second, **the**

capital is a bridge, not a burn: cumulative cash turns positive in month 9 at baseline and the business self-funds from there. Third, **most of it is inventory rather than losses** — see section 1. Operating losses total \$121 thousand across months one and two; the rest of the trough is stock paid for ahead of the revenue it produces.

Note also that the no-partner case needs \$1.14M — roughly 4 times the baseline requirement — and still is not cumulatively cash-positive at month 36. The partnership is not only a revenue lever; it is the main reason this is a small cheque rather than a venture round.

The real capital question is not how much, it is how long before you know. The four dials in section 4 and the retargeting incrementality test in section 5 are all measurable within sixty days of the supplement line going live. A staged commitment — fund the supplement launch, read the dials, then decide on the prescription build — converts an open-ended bet into a small, time-boxed one.

Where the money goes, baseline, first twelve months

Category	First 12 months	Note
Paid media, all three tiers	\$901k	Budget set at 24% of prior-month revenue, allocated cheapest tier first, subject to a payback gate.
Launch marketing and PR	\$60k	One-time, concentrated in the launch month.
Creative and content	\$106k	Production the brand controls, and the input that keeps tier-3 creative from fatiguing. Separate from partner compensation.
Team	\$251k	Deliberately small; the provider carries clinical operations and fulfilment. Floored at a share of revenue so it cannot stay flat as the business scales.
Legal and regulatory	\$96k	Front-loaded on purpose: entity structure, provider agreement review, formulary opinion, claims and label review, likeness and endorsement protocol, privacy.
Medical oversight	\$81k	Fractional medical director. Never compensated on prescription volume.
Provider fee	\$7k	\$600 a month. The whole point of the structure.
Certification, technology, insurance	\$48k	LegitScript, storefront and subscription tooling, product and professional liability cover.
Partner cash retainer	\$165k	Illustrative only. No terms offered or accepted.

Note the shape. Legal and regulatory spend is comparable to team cost in year one, and that is correct rather than excessive. In this category the compliance work is not overhead on the business; it is the part that makes the business sellable, bankable and insurable.

11. Illustrative partner economics

No terms have been offered, negotiated or accepted, and nothing on this page is an offer. It exists so the conversation can start from arithmetic rather than adjectives. The equity figures apply an illustrative revenue multiple and are not a valuation.

The structure modelled is a 5% royalty on net revenue, a monthly cash retainer, and an equity stake. The royalty is held identical across all three scenarios on purpose, so the scenarios compare business performance rather than deal terms.

	Conservative	Baseline	Aggressive
Cash retainer, per month	\$0	\$15,000	\$25,000
Royalty on net revenue	5.0%	5.0%	5.0%
Equity, fully diluted (reference)	10%	15%	20%
Cash retainer over 36 months	\$0	\$510k	\$850k
Royalty over 36 months	\$82k	\$1.4M	\$25.0M
Total cash over 36 months	\$82k	\$1.9M	\$25.8M
Royalty in year 3 alone	\$36k	\$743k	\$15.6M
Illustrative equity value at 3x year-3 revenue	\$213k	\$6.7M	\$186.9M
Incremental EBITDA the partnership generates	n/a (see note)	\$5.4M	n/a (see note)

Source / label: Partner_Economics sheet. The 3x revenue multiple is editable in the workbook and is illustrative: 2-5x net revenue is a commonly quoted range for profitable consumer health at scale, but multiples move with growth, margin and rate cycles. The incremental-EBITDA row is computed only for Baseline, because the no-partner counterfactual was built against Baseline drivers; the same comparison at other scenarios would need its own counterfactual column.

Five constraints any deal here has to respect

- **The partner promotes the brand and the supplement line, never specific compounded prescription products.** 503A compounders operate under advertising and promotion limits, prescription-drug promotion carries its own disclosure requirements, and celebrity promotion of a prescription product invites exactly the scrutiny a new brand cannot absorb. Separating what is promotable from what is prescribable is a design requirement.
- **Paid-media likeness rights have to be granted explicitly, not assumed.** Section 5's creative discount and a material part of the partnership's value in section 6 depend on running partner creative in paid advertising. Specify usage, territory, term, approval workflow and post-termination tail.
- **Every endorsement carries a clear and conspicuous disclosure of the material connection, on every post, in every format.** A founder's equity stake is a material connection. Given the FTC warning-letter history noted in section 8, this needs a written protocol and pre-publication review, not good intentions.
- **Clinical compensation may never vary with prescribing or revenue.** Partner compensation may be tied to revenue; the medical entity's and the clinicians' may not. Keeping those two facts in separate contracts is the entire point of the three-entity structure.
- **Category exclusivity has to be resolved before any announcement.** Earlier research in this project flagged a possible prior GLP-1-adjacent promotion by the proposed partner; the brand and

whether it was paid were not verified. A concurrent or recent competing endorsement is a diligence item, not a footnote.

12. The first ninety days

Everything above is a model. The purpose of the next ninety days is to replace its weakest assumptions with measurements, and to do the structural work that cannot be shortcut, before committing serious capital.

Weeks	Work	What it settles
1-2	Provider diligence. Put all twenty questions on the Provider_Diligence sheet in writing and get written answers. Priority: are per-consult fees inside the \$600, does the pharmacy bill per fill, who holds LegitScript and for which site, which pharmacies and which states, and 503A versus 503B for each product.	Whether the cost structure in this model is the real one. Nothing else is worth doing until this is answered.
1-4	Counsel engagement across six areas: entity structure and corporate practice of medicine, the provider and pharmacy agreements, product and claims review for the supplement line, the endorsement and disclosure protocol, PAID-MEDIA likeness rights, and auto-renewal and privacy compliance.	Whether the structure is defensible, and whether the section 5 media plan can legally use partner creative.
2-6	Supplement line build: contract manufacturer selection and run schedule, label and claims review, 3PL onboarding, storefront, subscription mechanics with a cancellation path as easy as the sign-up path.	The first-month shippable capacity number, which binds hard in the aggressive case.
2-6	Media infrastructure: LegitScript application, ad accounts, conversion tracking and server-side tagging, branded-search campaigns built and ready, custom and engagement audiences configured, and a partner-versus-generic creative A/B test designed before launch.	Whether the three tiers can actually be run, and the creative discount measured rather than assumed.
4-8	Formulary verification against the live FDA 503A bulk drug substances list, plus a written tracking obligation on the provider. Confirm nothing in the planned range is essentially a copy of an approved drug.	Which prescription products can legally exist. Do not take this from any summary, including this document.
6-10	Audience test. A small number of disclosed brand posts driving UTM-tagged traffic to a live storefront with real inventory, run alongside the paid tiers. Measure reach per post, click-through per reached impression, order conversion by source, branded-search volume lift, and first-cohort subscription attach.	Three of the four dials in section 4, plus the branded-search mechanism that underpins tier 1.
8-11	Retargeting holdout test. Suppress retargeting for a randomised share of the addressable pool and measure the difference.	The incrementality haircut in section 5, which decides whether a fifth of the media budget is well spent.
11-13	Read the data and decide. Re-run the model on measured drivers. Decide whether to fund the prescription build, extend the supplement-only test, or stop.	Whether this is the baseline case, the aggressive case, or the conservative case. You will know by month three, not month thirty.

The recommendation is unchanged, and the media work strengthens it: launch the supplement line first and alone. It is fifty-state legal, carries the better margin, is the only line the partner can promote, needs no prescriber or pharmacy, and it measures every growth dial and every media assumption for a fraction of the cost of the full build. If the numbers come in at baseline or better, the prescription line becomes a funded decision made on evidence rather than on a deck.

13. How these numbers were built, and what to distrust

Every figure in this document is a formula over the Assumptions sheet of outputs/09_ReserveClinic_Margin_and_Growth_Model.xlsx. There are no hard-coded results in the monthly grids. The workbook recalculates cleanly: 22,358 formulas across six scenario columns, zero errors. Change any driver and every number here moves.

One substantive correction from the previous draft, recorded here deliberately. That draft assumed a \$104,000 monthly fixed cost base — venture-scale team, legal and creative spend on top of a plug-and-play operation — which delayed break-even to month ten and inflated the capital requirement to \$803,000. The unit economics were unchanged and were never the constraint. Section 1 sets out the corrected cost base line by line. The lesson generalises: in a model like this the overhead assumption moves break-even more than any demand assumption does, so it deserves the same scrutiny as the growth drivers.

The four things most likely to be wrong

Note that the cost base is no longer on this list, because it has been corrected to the structure described. It is on the list of things to confirm in writing instead — see the provider diligence questions in section 12.

- **Audience conversion.** Reach per post and click-through are multiplied together and neither is measured. Section 4's grid shows the spread. Still the largest source of uncertainty in the document by a wide margin.
- **The celebrity creative discount.** No study quantifying the advertising lift from celebrity or recognisable-face creative was retrievable. The supporting evidence is only general — video creative 30-50% higher click-through, faces in visuals recommended. The month-one A/B test in section 12 is how this stops being an assumption.
- **Prescription retention.** The outside evidence points to worse retention than the model assumes at baseline. Section 8 shows where it stops working.
- **The provider's scope.** A \$600 flat fee is roughly one fifth of the low end of published market pricing for comparable turnkey infrastructure. The model assumes it covers software and administration only and carries every variable cost separately.

What could not be verified in this environment

- First-party analytics for any competitor (sessions, conversion rate, CAC, churn) — no traffic tool or ad library was reachable.
- Audited financials for any private peptide or celebrity supplement brand. IM8 and Gruns revenue figures are self-reported via press.
- Scott Disick's audience composition (US share, age, gender, health-category affinity) and any historical conversion performance on a paid endorsement.
- The identity, licences, contract terms, insurance, pharmacy network and LegitScript status of the \$600/month provider — no provider was named.
- Any study quantifying the advertising performance lift from celebrity or recognisable-face creative. The creative-discount driver is an analyst assumption supported only by general findings on video and faces in visuals.

- A branded-search conversion-rate multiplier benchmark, and any benchmark for branded-search volume generated per unit of social reach. Both are analyst assumptions.
- Whether the proposed partner's likeness rights extend to paid media. Organic posting rights and paid-usage rights are separately licensed, and health-category ad review plus LegitScript certification gate the paid use regardless.

Evidence base

The model's drivers rest on 20 documented benchmarks recorded in research/07_margin_growth_benchmarks.json and reproduced on the Benchmarks sheet of the workbook, each with its finding, how it is used, a confidence rating and its sources. Figures were taken from search-result summaries during sessions in which direct page loads were restricted; load-bearing figures were corroborated across two independent queries. Ranges are ranges, not measurements.

ID	Topic	Confidence
B01	Public telehealth gross margin (closest listed comparable)	High (company-reported)
B02	Telehealth marketing intensity	High (company-reported)
B03	DTC supplement gross margin and landed COGS	Medium (aggregator/advisory sources, not audited)
B04	DTC supplement CAC	Medium
B05	Supplement subscription churn	Medium
B06	Supplement AOV, LTV and LTV:CAC	Medium
B07	Compounded peptide programme pricing, US, 2026	Medium (marketing pages and category guides, not a price survey)
B08	Asynchronous consult cost and price	Medium
B09	Cold-chain fulfilment cost for peptides	Medium (vendor marketing pages)
B10	LegitScript healthcare merchant certification cost	Medium (third-party summaries of a schedule that changes; confirm live)
B11	Market rate for turnkey white-label telehealth infrastructure	Medium
B12	Category growth: peptide therapeutics (global)	Low (vendor market reports, wide disagreement)
B13	Category growth: dietary supplements	Low (vendor market reports)
B14	Celebrity-founded supplement brand outcomes (existence proof for the upside case)	Medium (press reporting, self-reported revenue)
B15	Instagram performance at the 10M-plus follower tier	Medium
B16	Proposed brand partner audience size	Medium (third-party trackers; not an audited or first-party figure)
B17	Prescription-programme retention reality check	Medium-High (peer-reviewed real-world evidence plus trade commentary)

ID	Topic	Confidence
B18	503A bulk substances list movement on peptides, 2026	Medium for the FDA actions, Low for the CJC-1295/ipamorelin Category 1 claim
B19	Retargeting versus cold prospecting economics, and the incrementality problem	Medium for the nominal efficiency figures, Medium-High for the incrementality warning (holdout-test based)
B20	Paid social creative performance, and the absence of a celebrity-specific benchmark	Medium for the general paid-social figures, None for celebrity-specific lift (no source found)

Status of the people named, restated

Scott Disick and Dr Michael Azziz are treated strictly as proposed participants. No public evidence connects either to this venture. No agreement exists. Nothing in this document implies endorsement, ownership or authorisation, and no part of it should be shared as though terms exist. Earlier research in this project could not verify a physician with the exact spelling 'Michael Azziz'; the closest candidate was identified at roughly 60% confidence and requires direct confirmation of identity, licensure and credentials before any use of the name.

Required professional review

This document identifies legal and regulatory issues; it is not legal, medical, tax or investment advice and must not be used as any of them. Before acting on any part of it, obtain review from qualified counsel in each of: FDA and drug-compounding regulation; state pharmacy law; corporate practice of medicine and fee-splitting; telehealth licensure and asynchronous prescribing; healthcare privacy including HIPAA, the FTC Health Breach Notification Rule and state health-data statutes; advertising, endorsement and claims substantiation; right of publicity and likeness licensing for paid media; consumer protection and auto-renewal including ROSCA and state analogues; product liability; and intellectual property. Clinical protocol design, formulary selection and patient safety are decisions for licensed clinicians, not for this model.